

Some pelvic pain does not come from the bladder, bowel or sex organs. It comes from the **muscles, joints and nerves** of the pelvis. Often the cause is a tight pelvic floor that will not relax. This pain is real, common and treatable, even when scans and lab tests are normal.

About This Condition

The pelvic floor muscles can get tight or go into spasm, like a knotted muscle anywhere else. Common features:

- Aching, pressure or burning in the pelvis, genitals, low back, hips or thighs
- Pain when **sitting** for a long time, during or after sex, or with bowel movements
- A frequent or urgent need to urinate, or a feeling of not emptying, caused by the tight muscles

What Causes It

Often several things add up: muscle tension, a past injury, surgery or childbirth, posture, and stress, which makes us clench. It can also happen along with other pain conditions, such as bladder pain or endometriosis. The pelvic floor is often too **tight**, not too weak. That is why Kegel squeezing exercises can make it worse.

Learn the Terms

Pelvic floor

The sling of muscles that holds up the bladder, bowel and other pelvic organs.

Tight pelvic floor

Muscles that stay clenched and cannot fully relax.

Dyspareunia

Pain with sex.

Trigger point

A sore muscle knot that can send pain to other areas.

Pelvic floor therapy

Physical therapy that relaxes and retrains these muscles.

Down-training

Learning to relax, not strengthen, a tight pelvic floor.

Why are my tests normal if I still hurt? This pain comes from muscles and nerves. It does not show up on scans or blood tests. A normal test is good news. It points to a treatable muscle problem, not something dangerous. Pain that does not show on a scan is still real.

How It's Diagnosed

- Questions about where the pain is, when it happens and what makes it worse
- A gentle exam that **checks the pelvic floor muscles** for soreness and tightness
- Simple tests, such as a urine test, to rule out other causes

How It's Treated

- 1 **Pelvic floor physical therapy** comes first. It uses hands-on release, stretching and relaxation ("down-training"), plus exercises at home.
- 2 **Self-care:** heat, gentle movement, deep breathing, and treating constipation.
- 3 **If pain continues:** numbing shots into sore muscle knots (trigger point injections), or talk therapy for pain.
- 4 **For some people:** Botox into the pelvic floor. This is an off-label use, and results in studies are mixed.

There is little proof that pain pills help this type of pain. Opioids are not recommended.

Living With It

- Notice when you clench your jaw, buttocks or pelvic floor, and let go.
- Use heat and gentle stretches. Avoid sitting on hard seats for long.
- Progress is slow, over weeks to months of therapy. Keep at it.

Call the office that is treating you if you have:

- New or much worse pelvic pain
- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Blood in your urine or stool

Go to an emergency department if:

- You have new numbness in the groin or inner thighs, or new leg weakness
- You suddenly cannot control, or cannot pass, urine or stool
- You cannot reach the office and are worried

Three Things to Remember

1. Pelvic pain often comes from tight muscles and nerves. It is real and treatable, even with normal tests.
2. Pelvic floor therapy that teaches the muscles to **relax** is the main treatment.
3. Improvement takes time. Keep up your home program.